

A new version of the document will be sent should new information become available

Our Ref: (T) JS/LB/[MED-ID]

[DATE]

Clinic Letter : [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division of Unscheduled Care

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE] Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME], [ADDRESS]. [POSTCODE]

DOB: [DATE], Hospital Number: [MED-ID], NHS Number: [NHS-NO]

Diagnoses:

Ulcerative colitis

Subtotal colectomy and ileoanal pouch formation

Short bowel syndrome

Bilateral DVTs on long-term warfarin

Partial hypoadrenalism

Epilepsy

Previous bacterial overgrowth

I reviewed this nice lady today in clinic who remains well. She had a brief episode of difficulty of her recent knee replacement in June at Salisbury Hospital, but has otherwise continued months of reasonable health. She states she is eating normally and her weight is maintained. She restricts her fluid intake and drinks 1000 millilitres of St Mark's a day. She is on a course of antibiotics for bacterial overgrowth at the moment. She has always had metronidazole for this and we often find it is useful to rotate these antibiotics to a different type, often co-amoxiclavate is beneficial, before rotating back to the metronidazole again. Her recent blood tests and urinalysis are satisfactory and I have simply asked to see her again in six months' time. If she has any problems she will contact us.

Yours sincerely

[PROVIDER-NAME]

Specialist Registrar in Gastroenterology